

Who to screen

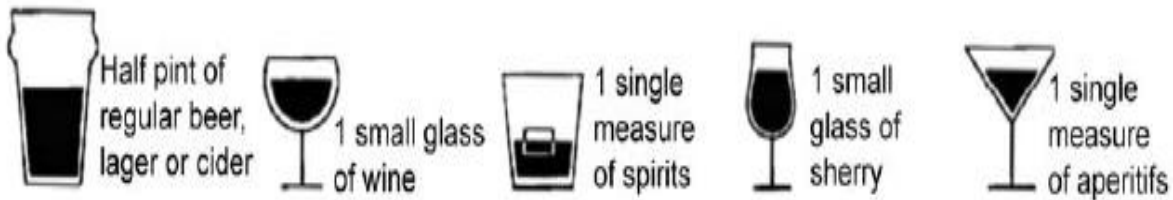
Alcohol screening should be an integral part of professional practice to identify those with alcohol use problems

- New patient registration
- When screening for other problems – e.g. at health check
- When monitoring chronic disease
- Those with relevant physical/mental health complaints
- During medication reviews
- Patients who have been assaulted
- Those at risk of self-harm
- When promoting sexual health
- During antenatal appointments
- Presenting with minor injuries or history of accidents/minor trauma/falls
- Incidental findings which may be relevant e.g.
 - blood results – raised LFTs, MCV, low platelet count
 - examination findings > 5 spider naevi, palmar erythema, hepatomegaly or splenomegaly
- Active request for help

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Audit C

This is one unit of alcohol...



...and each of these is more than one unit



AUDIT – C

Questions	Scoring system					Your score
	0	1	2	3	4	
How often do you have a drink containing alcohol?	Never	Monthly or less	2 - 4 times per month	2 - 3 times per week	4+ times per week	
How many units of alcohol do you drink on a typical day when you are drinking?	1 - 2	3 - 4	5 - 6	7 - 9	10+	
How often have you had 6 or more units if female, or 8 or more if male, on a single occasion in the last year?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	

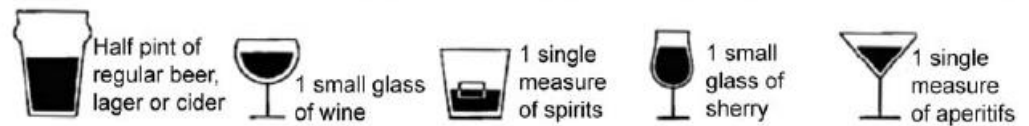
Scoring:

A total of 5+ indicates increasing or higher risk drinking.

An overall total score of 5 or above is AUDIT-C positive.



This is
one unit



...and each
of these is
more than
one unit



AUDIT	Scoring system					Your score
	0	1	2	3	4	
How often do you have a drink containing alcohol?	Never	Monthly or less	2 - 4 times per month	2 - 3 times per week	4+ times per week	
How many units of alcohol do you drink on a typical day when you are drinking?	1 - 2	3 - 4	5 - 6	7 - 9	10+	
How often have you had 6 or more units if female, or 8 or more if male, on a single occasion in the last year?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	
How often during the last year have you found that you were not able to stop drinking once you had started?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	
How often during the last year have you failed to do what was normally expected from you because of your drinking?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	
How often during the last year have you needed an alcoholic drink in the morning to get yourself going after a heavy drinking session?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	
How often during the last year have you had a feeling of guilt or remorse after drinking?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	
How often during the last year have you been unable to remember what happened the night before because you had been drinking?	Never	Less than monthly	Monthly	Weekly	Daily or almost daily	
Have you or somebody else been injured as a result of your drinking?	No		Yes, but not in the last year		Yes, during the last year	
Has a relative or friend, doctor or other health worker been concerned about your drinking or suggested that you cut down?	No		Yes, but not in the last year		Yes, during the last year	

Scoring: 0 – 7 Lower risk, 8 – 15 Increasing risk, 16 – 19 Higher risk, 20+ Possible dependence



NICE recommends using formal assessment tools to assess the nature and severity of alcohol misuse. See NICE CG115. NICE also recommends 'professional judgement' is used when interpreting AUDIT scores as some people are more vulnerable to the effects of alcohol e.g. low BMI, women, >65years and some ethnic groups who may not metabolise alcohol as effectively and polysubstance users.

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SEVERITY OF ALCOHOL DEPENDENCE QUESTIONNAIRE (SADQ-C)

NAME_____AGE_____No._____

DATE:_____

Please recall a typical period of heavy drinking in the last 6 months.

When was this? Month Year Please answer all the following questions about your drinking by circling your most appropriate response.

During that period of heavy drinking

1. The day after drinking alcohol, I woke up feeling sweaty.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

2. The day after drinking alcohol, my hands shook first thing in the morning.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

3The day after drinking alcohol, my whole body shook violently first thing in the morning if I didn't have a drink.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

4. The day after drinking alcohol, I woke up absolutely drenched in sweat.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

5. The day after drinking alcohol, I dread waking up in the morning.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

6 The day after drinking alcohol, I was frightened of meeting people first thing in the morning.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

7. The day after drinking alcohol, I felt at the edge of despair when I awoke.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

8. The day after drinking alcohol, I felt very frightened when I awoke.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

9. The day after drinking alcohol, I liked to have an alcoholic drink in the morning.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

10. The day after drinking alcohol, I always gulped my first few alcoholic drinks down as quickly as possible.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

11. The day after drinking alcohol, I drank more alcohol to get rid of the shakes.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

12. The day after drinking alcohol, I had a very strong craving for a drink when I awoke.

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

13. I drank more than a quarter of a bottle of spirits in a day (OR 1 bottle of wine OR 8 units of beers).

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

14. I drank more than half a bottle of spirits per day (OR 1.5 bottles of wine OR 15 units of beer).

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

15. I drank more than one bottle of spirits per day (OR 3 bottles of wine OR 30 units of beer).

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

16. I drank more than two bottles of spirits per day (OR 6 bottles of wine OR 60 units of beer)

ALMOST NEVER SOMETIMES OFTEN NEARLY ALWAYS

Imagine the following situation:

1. You have been completely off drink for a few weeks 2. You then drink very heavily for two days

How would you feel the morning after those two days of drinking?

17. I would start to sweat.

NOT AT ALL SLIGHTLY MODERATELY QUITE A LOT

18. My hands would shake.

NOT AT ALL SLIGHTLY MODERATELY QUITE A LOT

19. My body would shake.

NOT AT ALL SLIGHTLY MODERATELY QUITE A LOT

20. I would be craving for a drink.

NOT AT ALL SLIGHTLY MODERATELY QUITE A LOT

SCORE _____ CHECKED BY: ALCOHOL DETOX PRESCRIBED: YES/NO

NOTES ON THE USE OF THE SADQ

The Severity of Alcohol Dependence Questionnaire was developed by the Addiction Research Unit at the Maudsley Hospital. It is a measure of the severity of dependence. The AUDIT questionnaire, by contrast, is used to assess whether or not there is a problem with dependence.

The SADQ questions cover the following aspects of dependency syndrome:

- physical withdrawal symptoms
- affective withdrawal symptoms
- relief drinking
- frequency of alcohol consumption
- speed of onset of withdrawal symptoms

Scoring Answers are rated on a four-point scale:

Almost never- 0 , Sometimes -1, Often -2, Nearly always - 3

A score of 31 or higher indicates "severe alcohol dependence".

A score of 16 -30 indicates "moderate dependence"

A score of below 16 usually indicates only a mild physical dependency.

A chlordiazepoxide detoxification regime is usually indicated for someone who scores 16 or over.

It is essential to take account of the amount of alcohol that the patient reports drinking prior to admission as well as the result of the SADQ.

There is no correlation between the SADQ and such parameters as the MCV or GGT.

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Brief Intervention

Evidence – There is consistent evidence from a large number of studies that brief intervention in primary care can reduce alcohol consumption and episodes of binge drinking in hazardous drinkers, for periods lasting up to a year. The effectiveness of brief intervention is well-established.

The characteristics of brief advice are:

- Opportunistic – delivered in empathetic and non-judgemental way
- Based on advice covering potential harm and the benefits of reducing/stopping, barriers to change
- Practical suggestions on how to reduce alcohol consumption including community support networks AA , smart recovery
- Leads to a set of goals
- With or without formal follow-up
- Up to 10 minutes in duration

This type of brief advice is most effective for those drinking at hazardous levels i.e. 8-15 on AUDIT.

Brief Advice is not designed to treat those with alcohol dependence, which generally requires greater expertise and more intensive clinical management

Brief intervention has been defined as having six essential elements summarised by the acronym **FRAMES** (Miller and Sanchez, 1993).

- **Feedback:** provide feedback on the patient's risk for alcohol problems
- **Responsibility:** highlight that the individual is responsible for change
- **Advice:** advise reduction or give explicit direction to change
- **Menu:** provide a variety of options for change
- **Empathy:** emphasise a warm, reflective and understanding approach
- **Self-efficacy:** encourage optimism about changing behaviour

Motivational interventions – key elements

- Help people to recognise problems or potential problems related to their drinking
- Help to resolve ambivalence and encourage positive change and belief in the ability to change
- Being persuasive and supportive rather than argumentative and confrontational

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This is one unit...



Half pint of regular beer, lager or cider



1 very small glass of wine(9%)



1 single measure of spirits



1 small glass of sherry



1 single measure of aperitifs

How many units did you drink today?

...and each of these is more than one unit



2

A pint of "regular" beer, lager or cider



3

A pint of "strong"/"premium" beer, lager or cider



1.5

Alcopop or a 275ml bottle of regular lager



2

440ml can of "regular" lager or cider



4

440ml can of "super strength" lager



3

250ml glass of wine (12%)



9

Bottle of wine (12%)

Risk	Men	Women	Common Effects
Lower Risk	No more than 14 units per week spread over 3 or more days	No more than 14 units per week spread over 3 or more days	•Increased relaxation •Sociability •Some possible evidence of protective effects for heart disease in small amounts •Small increased risk of some cancers
Increasing Risk	Over 14 units per week	Over 14 units per week	Progressively increasing risk of: •Low energy •Depression •Insomnia •Impotence •Injury & social problems •Alcohol dependence •High blood pressure •Liver disease •Cancer & many more
Higher Risk	28 units or more per week on a regular basis	28 units or more per week on a regular basis	

Download this 'IBA tool' from www.alcohollearningcentre.org.uk or www.alcoholiba.com

There are times when you will be at risk even after one or two units. For example, with strenuous exercise, operating heavy machinery, driving or if you are on certain medication.

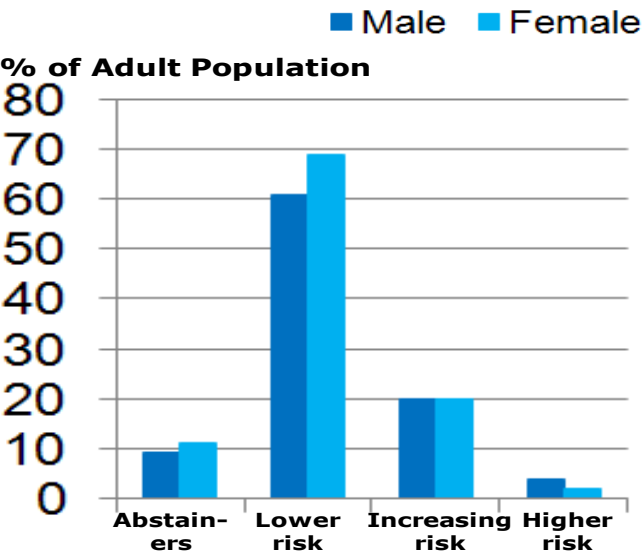
If you are pregnant or trying to conceive, it is recommended that you avoid drinking alcohol. But if you do drink, it should be no more than 1-2 units once or twice a week and avoid getting drunk.

Your screening score suggests you are drinking at a rate that increases your risk of harm and you might be at risk of problems in the future.

What do you think?



What's everyone else like?



Making your plan

- When bored or stressed have a workout instead of drinking
- Avoid going to the pub after work
- Plan activities and tasks at those times you would usually drink
- When you do drink, set yourself a limit and stick to it
- Have your first drink after starting to eat
- Quench your thirst with non-alcohol drinks before and in-between alcoholic drinks
- Avoid drinking in rounds or in large groups
- Switch to low alcohol beer/lager
- Avoid or limit the time spent with "heavy" drinking friends

The benefits of cutting down Psychological/Social/Financial

- Improved mood
 - Improved relationships
 - Reduced risks of drink driving
 - Save money
- Physical
- Sleep better
 - More energy
 - Lose weight
 - No hangovers
 - Reduced risk of injury
 - Improved memory
 - Better physical shape
 - Reduced risk of high blood pressure
 - Reduced risk of cancer
 - Reduced risks of liver disease
 - Reduced risks of brain damage

What targets should you aim for? Guidelines suggest that both men and women should aim for no more than 14 units per week.

These should be spread out over 3 days or more and single occasion amounts should be limited.

These guidelines were reviewed in 2016 following new evidence that any amount of alcohol increases risk of some cancers.

What is your personal target?

This brief advice is based on the "How Much Is Too Much?" Simple Structured Advice Intervention Tool, developed by Newcastle University and the Drink Less materials originally developed at the University of Sydney as part of a W.H.O. collaborative study.



Extended brief intervention is conducted by trained professionals

For those who:

- have not responded to brief interventions with hazardous drinking
- are harmful drinkers
- are suspected of being moderately dependant on alcohol but refuse referral for assisted withdrawal
- would benefit for other reasons such as they wish for further input.
-

EBI takes the form of motivation interview or motivational enhancement therapy and lasts 20-30mins

It helps individuals to address their alcohol use and to reduce the amount they drink to low risk levels or to abstinence.

Up to 4 sessions are given and patients should be followed up by the alcohol worker or their GP

If ineffective referral to a specialist alcohol treatment service should be considered

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Patient leaflets

Alcohol – recommended safe limits

<http://www.patient.co.uk/health/Recommended-Safe-Limits-of-Alcohol.htm>

Alcohol and sensible drinking

<http://www.patient.co.uk/health/Alcohol-and-Sensible-Drinking.htm>

Alcohol and problem drinking

<http://www.patient.co.uk/health/Alcoholism-and-Problem-Drinking.htm>

Alcohol and liver disease

<http://www.patient.co.uk/health/alcohol-and-liver-disease>

Alcohol detoxification

<https://patient.info/healthy-living/alcohol-and-liver-disease/alcoholism-and-problem-drinking>

Useful links

Alcoholic anonymous – 0800 917 7650

<http://www.alcoholics-anonymous.org.uk/>

Al-anon Family Groups <https://www.al-anonuk.org.uk/> – support for families/friends of alcoholics Helpline 0800 0086 811

National Association for Children of Alcoholics - provides information, advice and support for these vulnerable children and people concerned for their welfare – 0800 358 3456

www.nacoa.org.uk

Down your drink – online programme to reducing drinking

www.downyourdrink.org.uk

Drinkline – National drink helpline 0300 123 110

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Pregnancy and alcohol

Current Department of Health & Social Care guidance <https://www.gov.uk/government/publications/alcohol-consumption-advice-on-low-risk-drinking> is that drinking during pregnancy is not recommended. Anyone consuming alcohol during pregnancy should be advised not to do so, or if they choose to continue then should keep their drinking to the lowest possible level.

The safest approach is not to drink alcohol at all if pregnant, planning pregnancy or breastfeeding.

Although the risk of harm to the baby is low with small amounts of alcohol before becoming aware of the pregnancy, there is no 'safe' level of alcohol to drink when pregnant.

Refer patient to specialist alcohol service if they are pregnant and drinking >2 units a day.

Risk to fetus

- Fetal growth and development problems
- Increased risk of miscarriage
- Increased risk of structural malformation
- Fetal alcohol syndrome/fetal alcohol syndrome disorder

Patient resources

<http://www.patient.co.uk/health/fetal-alcohol-syndrome>

<https://www.rcog.org.uk/globalassets/documents/patients/patient-information-leaflets/pregnancy/pi-alcohol-and-pregnancy.pdf>

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Thiamine

See [RMOc Position Statement: Oral vitamin B supplementation in alcoholism](#) (A)

Deficiency is common in alcohol drinkers due to poor diet, poor absorption secondary to gastritis and high demand for the vitamin as it is a coenzyme in alcohol metabolism. Thiamine deficiency can cause Wernicke's Encephalopathy (WE) (reversible with thiamine supplements), which if not treated can result in Korsakoff's syndrome and irreversible brain damage.

- In line with NICE guidance, oral thiamine should be prescribed for the prevention of WE to harmful or dependent drinkers in whom any of the following apply:
 - They are malnourished or at risk of malnourishment
 - They have decompensated liver disease
 - They are in acute withdrawal
 - Before and during a planned medically assisted alcohol withdrawal
- The recommended dose is 200 to 300 mg daily in divided doses.
- Thiamine should be continued for as long as malnutrition is present and/or during periods of continued alcohol consumption.
- Following successful alcohol withdrawal, thiamine should be continued for 6 weeks. If after this time the patient remains abstinent and has regained adequate nutritional status, thiamine should be discontinued. Thiamine should be restarted if the patient starts drinking again.
- Continuing need for thiamine should be reviewed at appropriate intervals which may depend on individual patient circumstances.

Adverse effects of thiamine include mild gastrointestinal side effects-nausea, vomiting, diarrhoea and abdominal pain.

IM/IV Pabrinex® is used in hospital for those with poor health and severe malnutrition undergoing detox

Vitamin B complex preparations and other vitamin supplements are NOT recommended unless otherwise clinically indicated which is rare.

(A): <https://www.sps.nhs.uk/articles/rmoc-position-statement-oral-vitamin-b-supplementation/>

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Suitability for Community detox <75yrs

GPs with appropriate training/experience can undertake community alcohol detoxification (assisted withdrawal) in suitable patients < 75 years (chlordiazepoxide only).

Effective and safe treatment for patient with mild to moderate withdrawal symptoms

Consider in those drinking 15-30 units /day and /or scoring <30 SADQ (**Primary Care – consider in those with SADQ <16 unless able to see daily**)

There should be no history of epilepsy, seizures or delirium tremens or other significant comorbidities.

Inpatient /residential detox is advised if the patient

- Drinks over 30 units a day
- Has a SADQ score >30
- Is confused or has hallucination
- Has a history of previous complicated withdrawal e.g. delirium tremens/seizures, uncontrollable withdrawal symptoms
- Had epilepsy or history of fits
- Is vulnerable – homeless, elderly, learning disability or cognitive impairment, undernourished
- Has severe vomiting or diarrhoea
- Is at risk of suicide
- Has a previously failed community withdrawal programme
- Has significant physical or psychiatric co-morbidity
- Has multiple substance misuse
- Has a home environment unsupportive of abstinence

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Detox – confirm abstinence by checking for alcohol on breath or by using a breathalyser.

Monitor patient every 1-2 days (BP, pulse, respiratory rate, breath alcohol concentration(BAC), physical and mental state) and prescribe no more than 2 days supply at a time.

Formal measure of withdrawal symptoms CIWA-Ar <https://www.mdcalc.com/ciwa-ar-alcohol-withdrawal>

Adjust dose if severe withdrawal symptoms or over sedation.

Chlordiazepoxide –

- **Do not use if severe liver disease or >75yrs or pregnant.**
- Refer pregnant patients to specialist alcohol services for detox. Manufacturer of chlordiazepoxide contraindicates use in pregnancy, however UK Teratology Information Service (UKTIS) advice (<https://www.medicinesinpregnancy.org/upload/200522.pdf>) states that “The benefits of treating pregnant women (at any stage of pregnancy) experiencing acute alcohol withdrawal with chlordiazepoxide likely outweigh the risks”.
- Prescribe every 1-2 days. In primary care a tapered fixed –dose regimen is used with regular monitoring every 1-2days. Reducing to zero over 7-10 days. Other settings where a higher degree of supervision is available may use symptom triggered therapy where a person is monitored.

Chlordiazepoxide reducing dose regimes based on SADQ scores on day 1

Table 1. Grey fields - regimens to be given by specialists only.

Daily Alcohol Consumption	15-25 units		30-40 units		50-60 units
Severity of dependence	Mild/Moderate: SADQ Score <30		Severe: SADQ Score 30-40		Very Severe: SADQ score 40-60
Day 1	15mg qds	25mg qds	30mg qds	40mg qds	50mg qds
Day 2	10mg qds	20mg qds	25mg qds	35mg qds	45mg qds
Day 3	10mg tds	15mg qds	20mg qds	30mg qds	40mg qds
Day 4	5mg tds	10mg qds	15mg qds	25mg qds	35mg qds
Day 5	5mg bd	10mg tds	10mg qds	20mg qds	30mg qds
Day 6	5mg nocte	5mg tds	10mg tds	15mg qds	25mg qds
Day 7		5mg bd	5mg tds	10mqds	20mg qds
Day 8		5mg nocte	5mg bd	10mg tds	15mg qds
Day 9			5mg nocte	5mg tds	10mqds
Day 10				5mg bd	10mg tds
Day 11				5mg nocte	5mg tds
Day 12					5mg bd
Day 13					5mg nocte

Additional information:

Oxazepam/lorazepam – may be used in specialist services for patients with severe liver impairment. **Not** for prescribing in primary care.

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Relapse prevention medicines: Specialist initiation. Prescribing in primary care can be continued in primary care once patient stabilised, care plan is established and GP is in agreement with the plan.

Acamprosate:

- Reduces intensity of and response to cues and triggers to drinking
- For patients struggling to maintain abstinence and who describe anxiety as a feature of their difficulties in remaining sober. If a patient describes "craving" as a desire to get a "high or buzz" from alcohol, acamprosate is less likely to be effective
- Started as soon as abstinence is achieved, can be continued for up to 6 months, or longer if patient is benefiting. Stop acamprosate if drinking persists for 4-6 weeks after starting the drug. Seek specialist advice for treatment beyond 12 months. **Treatment is recommended as an adjunct to psychosocial interventions**

See SPC for acamprosate (<https://www.medicines.org.uk/emc/product/986/smpc>) for further prescribing information including dosing, contra-indications and side-effects.

Naltrexone:

- Reduces and suppresses cravings for alcohol
- Licensed for use as part of a comprehensive programme of treatment against alcoholism to reduce the risk of relapse, as support treatment in abstinence and to reduce the craving for alcohol.
- Also licensed for use as an additional therapy within a comprehensive treatment program including psychological guidance for detoxified patients who have been opioid-dependent.
- Patients taking oral naltrexone should stay under supervision, at least monthly, for 6 months, and at reduced but regular intervals if the drug is continued after 6 months. At 12 months the treatment should be discussed with the patient, and a stop date agreed on
- Treatment must begin **only when any opioid has been discontinued** for a sufficiently long period of time

See SPC for naltrexone (Adepend®) (<https://www.medicines.org.uk/emc/product/3559/smpc>) for further prescribing information including dosing, contraindications and side-effects.

Disulfiram:

- Is a deterrent or alcohol sensitising drug. If alcohol is consumed by an individual taking disulfiram within 10-15 minutes they can experience: severe headache, violent flushing, palpitations and tachycardia, nausea and vomiting, hypotension, collapse, cardiac arrhythmias. The severity of the reaction varies between individuals some having severe and occasionally life-threatening reactions, others having mild or no reaction at standard doses (these latter individuals may require higher doses)
- As an adjuvant in the treatment of selected and cooperative patients with drinking problems accompanied by appropriate supportive treatment
- Must be stopped if patient restarts consumption of alcohol. Patient needs to be reviewed every 6 months at least and treatment duration discussed with the specialist

See SPC for disulfiram (<https://www.medicines.org.uk/emc/product/11168>) for further prescribing information including dosing, contraindications and side-effects.

Medicines used to reduce alcohol consumption.

Nalmefene:

- Nalmefene is recommended by NICE (TA 325 - <https://www.nice.org.uk/guidance/ta325>) as an option for reducing alcohol consumption, for people with alcohol dependence who have a high drinking risk level without physical withdrawal symptoms **and** who do not require immediate detoxification.
- Nalmefene should only be initiated by a specialist in conjunction with continuous psychosocial support focused on treatment adherence and reducing alcohol consumption. Can be continued in primary care.

See SPC for nalmefene (<https://www.medicines.org.uk/emc/product/3051>) for further prescribing information including dosing, contraindications and side-effects.

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Driving Advice

It is the driver's responsibility to contact the DVLA if they are persistently misusing alcohol or are dependent and it is against the law not to do so. This will result in revocation of their licence.

At follow up check if the patient has informed the DVLA explaining that you will have to inform the DVLA if they refuse to in order to protect them and others at risk from this behaviour.

See chapter 5 of the DVLA Guide of Medical Standards of Fitness to Drive https://www.gov.uk/government/uploads/system/uploads/attachment_data/file/312882/aagv1.pdf

Consider contacting your medical defence union for advice.

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Camden Drug and Alcohol Services Email: Camden.iCAS@cgl.cjsm.net

Recovery in Camden Guide: <https://gp-website-cdn-prod.s3.amazonaws.com/pathway-downloads/1560513720-552ad9866dc4f3604675a40e5b1e1e43.pdf>

Enable Enfield Drug & Alcohol Service T: 0208 379 6010 Email: beh-tr.enable@nhs.net

Website – <https://www.beh-mht.nhs.uk/services/enable-enfield-alcohol-and-drug-service/189>

Claverings, 12 Centre Way, Edmonton, London, N9 0AH
Vincent House, 2e Nags Head Road Ponders End, EN3 7FN

Drop in – visit Clavering site for a face to face discussion without an appointment on a Monday to Friday between 10am – 4pm.

Manager: Serena McCabe serena.mccabe@nhs.net

Better Lives - Islington's Drug and Alcohol Service Email: betterlivesreferrals@candi.nhs.uk

Website – <https://www.candi.nhs.uk/our-services/better-lives-islington-drug-and-alcohol-service>

99-101 Seven Sisters Road, N7 7QP T: 0203 317 6099
309 Gray's Inn Road, WC1X 8QS T: 0203 317 6650
28b King Henry's Walk, N1 4NX T: 0203 317 7277

Islington's YP Drug and Alcohol Service

Website – <https://directory.islington.gov.uk/kb5/islington/directory/service.page?id=8GlgHdgC3Zl>

NOTE: Camden and Islington Public Health pay for licenses for the Lower My Drinking App: <https://www.lowermydrinking.com/>. Support and advice can also be found on: <https://oneyouislington.org/topic/drink-less-2/>.

Drug and Alcohol Service - Barnet T: 0300 303 2866 Email: Barnet.info@cgl.org.uk

Website – <https://www.changegrowlive.org/barnet>

Ground Floor, Dennis Scott Unit
Edgware Community Hospital, Burnt Oak Broadway
Edgware HA8 0AD

Open from 9:30 - 17:00 Monday – Friday

Haringey Action on Alcohol (HAGA) Service T: 0208 801 3999 Email: harccg.haga@nhs.net

Website – <https://www.haga.co.uk/haringey-services/>

HAGA Alcohol Service, 590 Seven Sisters Road, London, N15 6HR

Opening times: 9:00am – 5pm Monday, Wednesday, Thursday, Friday. 09:00 – 7:00pm Tuesdays.

HOSPITAL LIAISON SERVICES IN THE FIVE BOROUGHES

UCLH (Camden and central London)

(Long standing) Alcohol and Drug Liaison service

Lead Nurse Kate Robinson

Contact: 0203 456 7890 can for alcohol liaison

Hospital liaison service based in University College hospital in Camden

Offers: Specialist advice, assessment signposting and links to alcohol services across all London boroughs

Whittington (Based in Islington)

In reach Hospital Liaison service provided by Better Lives

Better Lives is Camden & Islington Mental health Trust

Lead : Moya Forthyse Drugs and Alcohol

Tel : 0203 317 6650

Email: Betterlivesreferrals@candi.nhs.uk

Offers: Specialist advice, assessment and signposting and links to alcohol and drug services across London.

Royal Free Hospital (Camden Based)

Alcohol and Drug Liaison

Lead Nurse : Jeff Fernandez

Tel: 0207 794 0500 ext 34003

Email: jfernandez@nhs.net

Offers assessment and signposting to services across London.

Also an out-patients service to help hard to reach clients integrate into mainstream services

Also offers community alcohol detoxification in ambulatory services for suitable patients

North Middlesex Hospital (Enfield): No alcohol liaison resource

St Mary Hospital Based in Paddington

Alcohol Liaison Service

Lead Nurse: Not known

Offers assessment, expert advice, and signposting to appropriate services for alcohol across London and wider if appropriate.

MENTAL HEALTH SERVICES FOR THE FIVE BOROUGHES

Barnet Enfield and Haringey Mental health trust

Offering comprehensive mental Health services across the boroughs of Barnet, Enfield and Haringey

Has a dual Diagnosis specialist post.

Multi-site service.

Main base St Ann's hospital

Tel: 0208 702 3000 (Main switch board)

Camden and Islington NHS Mental Health foundation Trust Multi Site service

Main Base St Pancreas hospital

Tel: 0203 317 3500 (Main switchboard)

Crisis Line 0203 917 3333

No specialist Dual diagnosis post

Do have access to a drug and alcohol service provide by the Trust

South/North Camden Drug service

Tel: 0203 317 6000

email: camdensms.referrals@candi.nhs.uk

Better Lives Islington drug and alcohol service

Tel: 0203 317 6650

email: betterlivesreferrals@candi.nhs.uk

Tavistock and Portman NHS Foundation trust

Offers mainly psychological therapies and interventions with access to psychotherapy and similar interventions

Tel: 0207 435 7111 (Main switchboard)